

2025

**HIV Corrective
Data Quality Audit
Executive Summary**

Strategic Information (SI) Data Quality Audit Report
HIV/STI/TB Unit, Health Promotion & Protection Branch



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Quality Audit
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2025

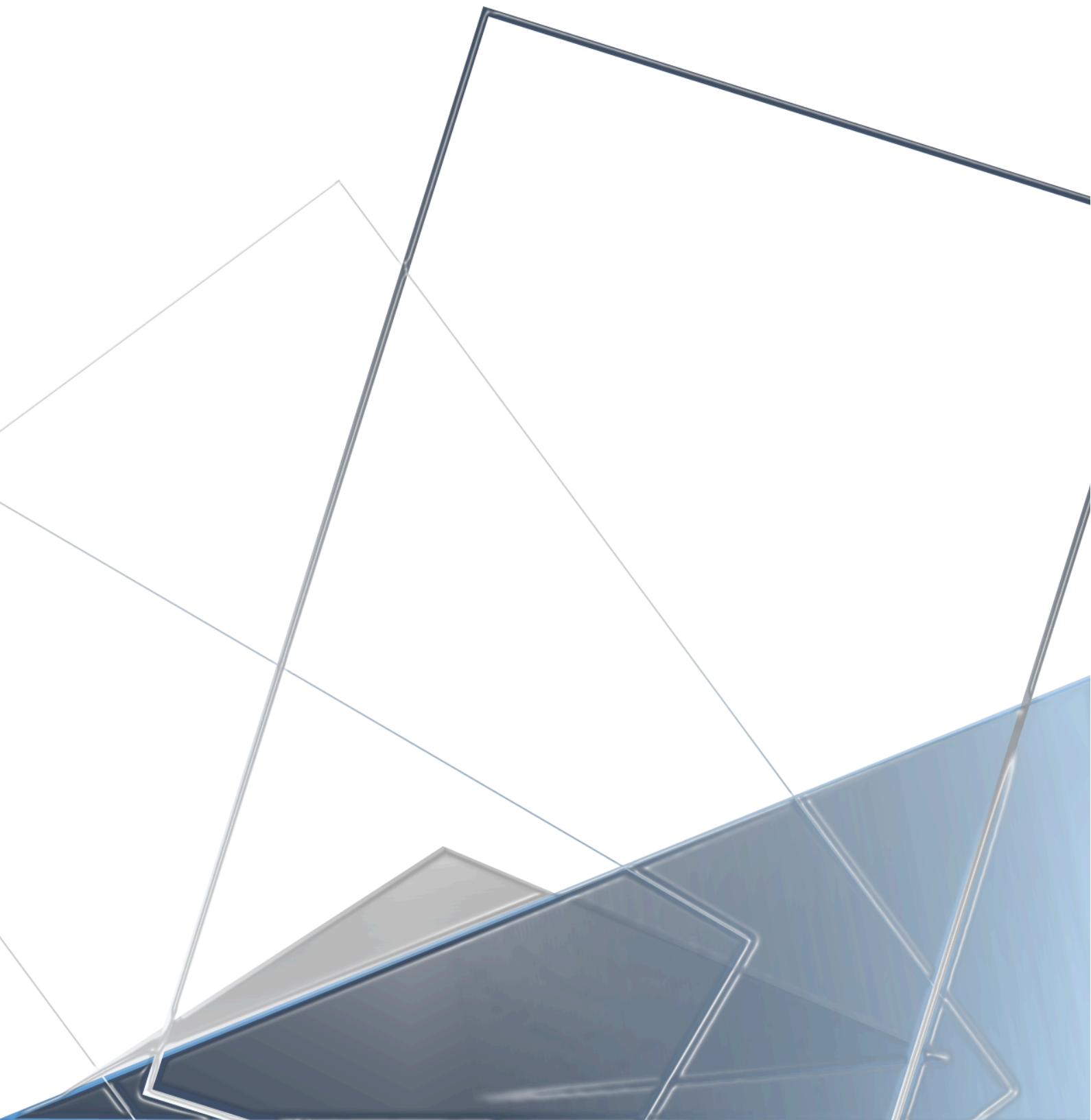


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In 2024, the national overall Data Quality Audit (DQA) score was 85%. Consequently, sites that scored 85% or below were selected for inclusion in the 2025 Corrective Data Quality Audit we conducted between May 8 and July 11, 2025.

A total of 1,317 medical records were reviewed and audited during this period. Twelve (12) indicators were examined for historical HIV care information, demographic information, retention in care and treatment information, viral load information and CD4 information (CD4 date and results were first formally audited during this audit; however, it did not contribute towards the overall score). Additionally, the organization, data collection, preparation, and storage of reports were assessed through interviews with the site staff.

The 2025 Corrective Data Quality Audit results indicate a significant decline in overall data quality performance compared to 2024. The corrective DQA score decreased to 68%, reflecting an 8% reduction from 76% in 2024, although the performance remained within the “Acceptable” compliance category. Despite this classification, the decline signals systemic challenges in maintaining accurate, complete, and secure patient data.

The largest decline occurred in the preparation and secure storage of reports, indicating increased risk to patient confidentiality and weaker compliance with the Data Protection Act. This decline is due to inconsistent password protection of electronic files and inadequate safeguards for sensitive patient information, making data governance the most urgent area requiring intervention.

Closely linked to this is the decline in accuracy of high-impact clinical and programmatic indicators, particularly: Next appointment dates, Viral load dates and results, Last clinic visit dates, and ARV treatment dates. These inaccuracies directly affect patient tracking, retention in care, and treatment monitoring, posing risks to both clinical management and programme reporting.

Additionally, the lowest score in the proportion of fully matched medical records reflects broader systemic issues in record reconciliation between paper files and TSIS, further compounding data reliability concerns.

Performance across entities remained uneven, with only half of the audited entities performing above the national average. Independent and civil society sites continued to outperform regional health authorities, suggesting differences in data management practices and resource capacity.

Overall, the audit underscores an urgent need to strengthen data accuracy for key clinical indicators and significantly improve data protection and reporting practices. Without targeted corrective actions in these areas, declines in data quality may continue to undermine programme performance monitoring and clinical decision-making.

Overall Results For Corrective Data Quality Audit

The overall corrective DQA score in 2025 was 68%, a 8% decline from 2024. Compared to 2024, the proportion of medical records fully matched with TSIS declined by 1%, the accuracy of data entered in the tool declined by 3%, the evaluation of the organizational component declined by 6%, the process of data collection improved by 2%, and the preparation and storage of reports declined by 33%. These results underscore the need for interventions to improve data protection of patient information and compliance with the Data Protection Act (Figure 1).

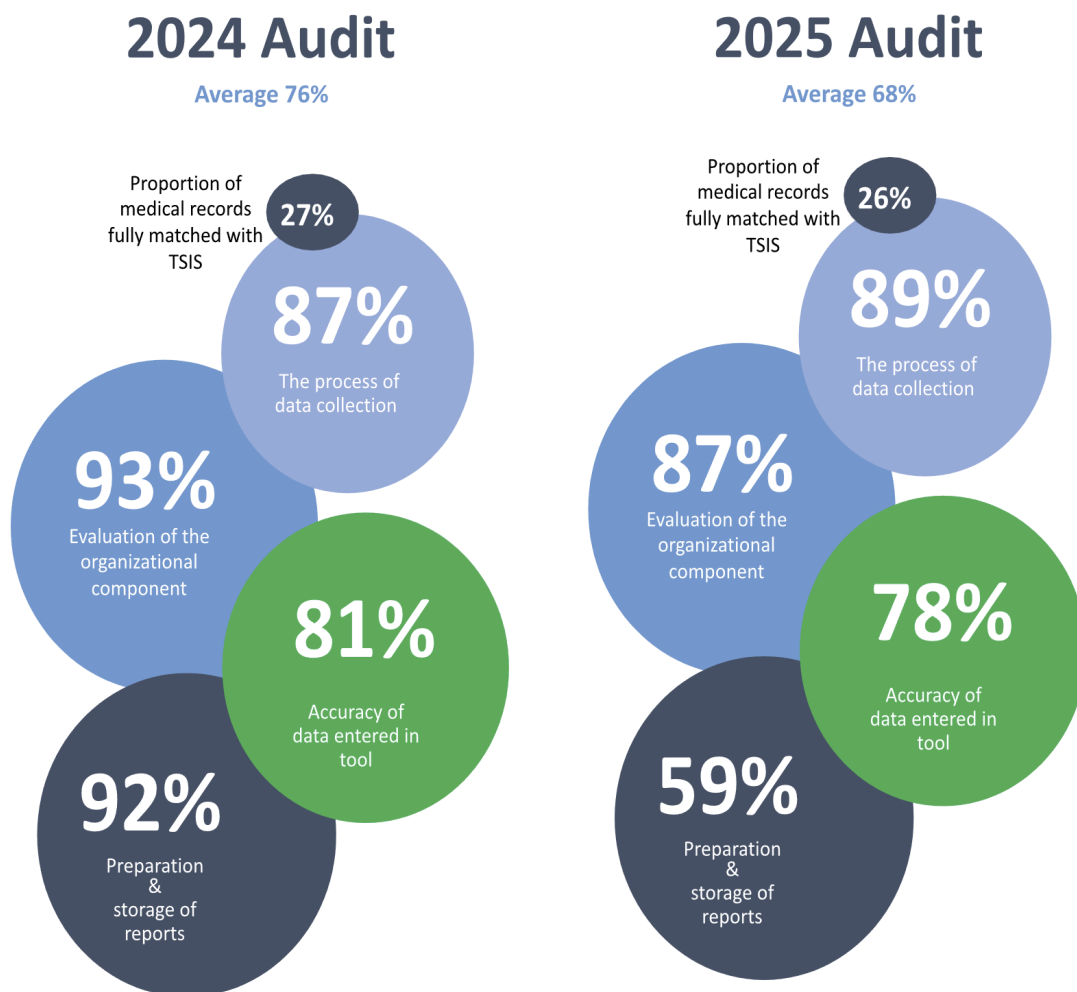


Figure 1: Corrective Data Quality Audit - Evaluation of Items

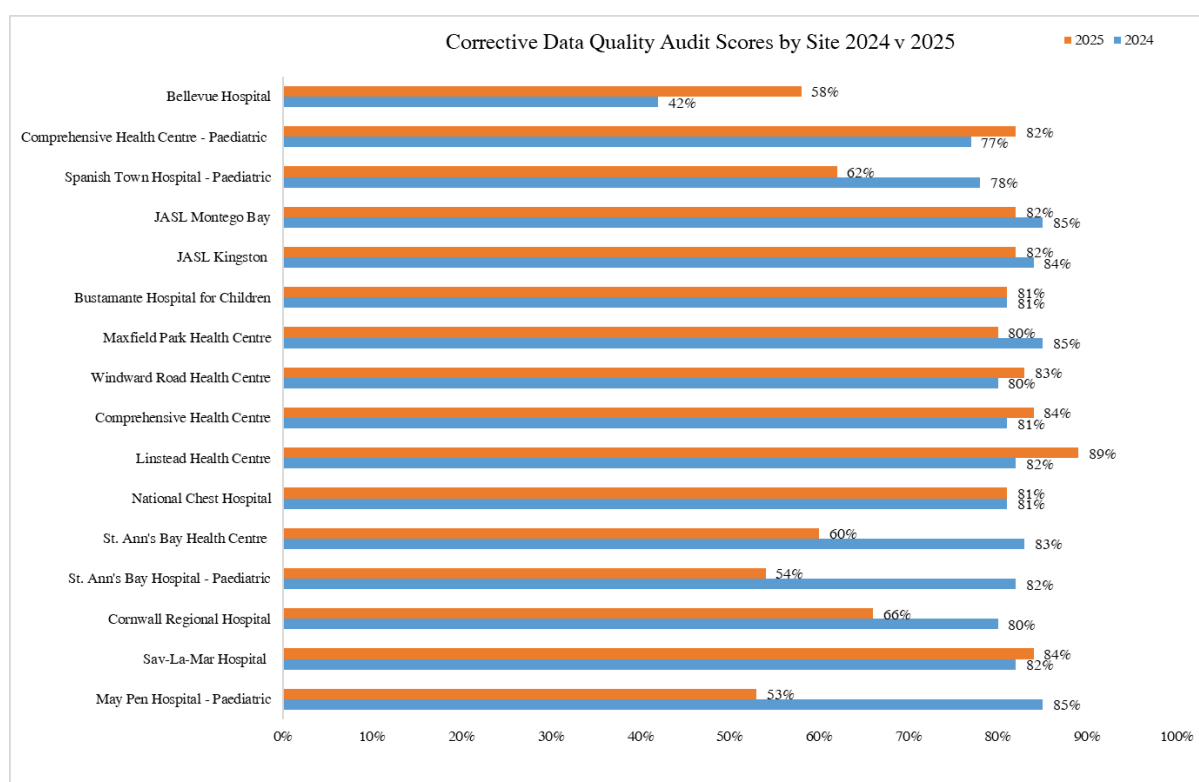


Figure 2: Compliance Scores by Site 2024 v 2025

Figure 2 shows the site-level results, which demonstrate a general decline in data quality performance from 2024 to 2025, with most facilities recording lower scores in 2025. The most pronounced decline was observed at paediatric and hospital-based sites. May Pen Hospital – Paediatric, St. Ann’s Bay Hospital – Paediatric, and St. Ann’s Bay Health Centre experienced steep drops. Spanish Town Hospital – Paediatric also recorded a notable decline.

Some sites showed little improvement, though performance remained suboptimal. Bellevue Hospital improved from a very low baseline but continued to be the lowest-performing site overall, indicating persistent structural and organizational data quality issues.

Independent and civil society sites, particularly the Jamaica AIDS Support for Life (JASL) locations, remained among the stronger performers, although both Kingston and Montego Bay sites experienced slight declines and fell below the 85% threshold. Linstead Health Centre stood out as one of the better-performing sites in 2025.

Overall, the site-level scores highlight uneven performance and widening gaps between facilities, with the sharpest declines occurring in paediatric and hospital settings. These findings emphasize

the need for targeted, site-specific corrective actions, particularly focused on strengthening clinical indicator accuracy, record reconciliation, and routine data quality monitoring.

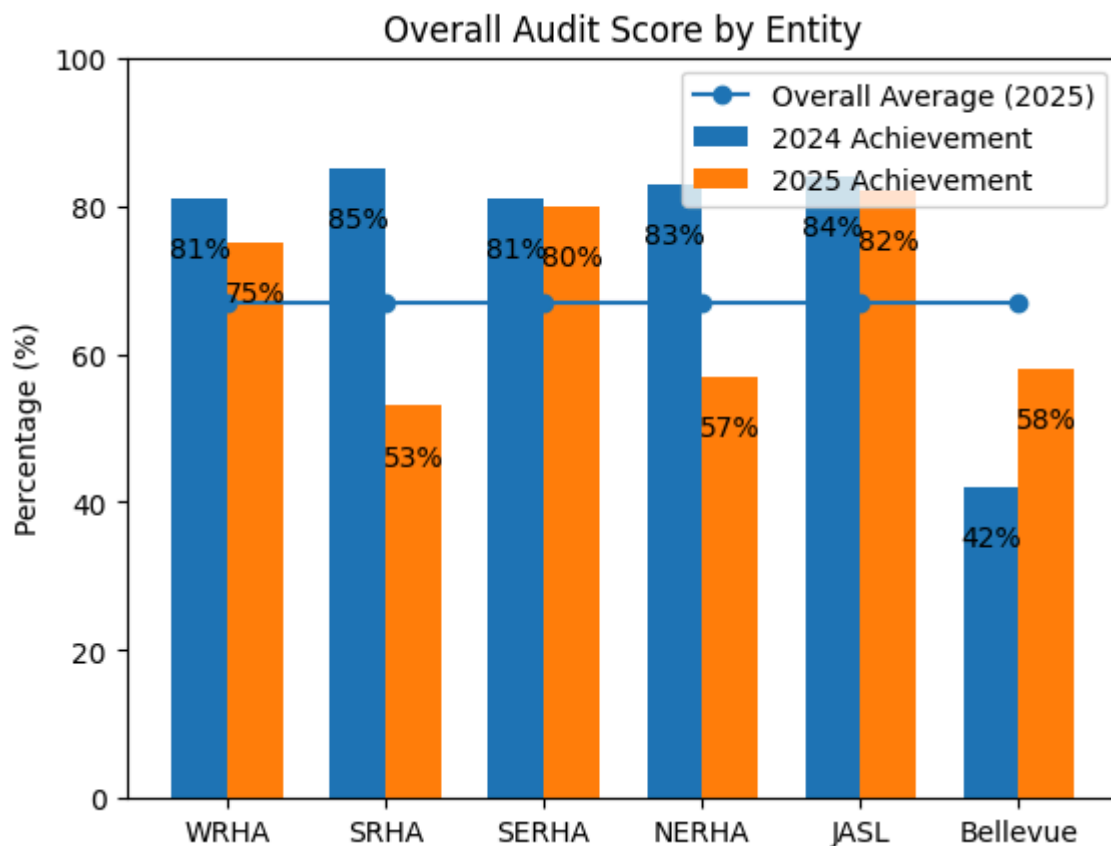


Figure 3: Overall DQA Score by Entity, 2024 v 2025

Figure 3 highlights that three of the six entities audited had an overall DQA score above the overall average of 68%. Seventeen per cent (1/6) of the entities achieved a “**Good**” score on the DQA compliance scale. The highest performer was JASL (82%), and the lowest was Bellevue (58%).

Matching Indicators

Overall data matching declined from 81% in 2024 to 76% in 2025, indicating persistent data quality gaps (Figure 4).

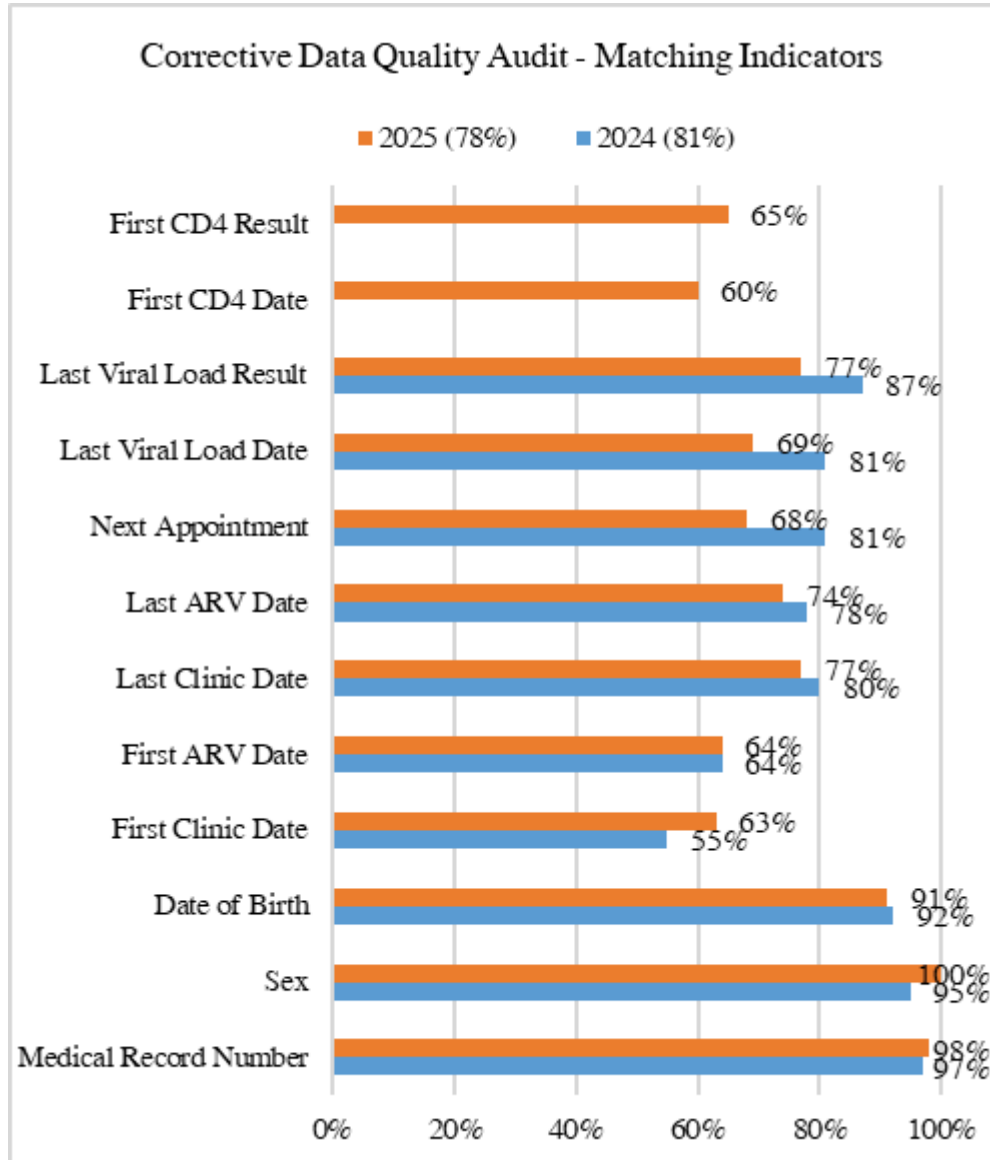


Figure 4: Overall Corrective Data Quality Audit - Matching Indicators (2024 v 2025)

Key strengths in data matching

- Sex remained high and improved to 100% in 2025 (from 95%).
- Medical Record Number stayed consistently strong (98% in 2025, 97% in 2024).
- Date of Birth remained high with only a slight dip (91% in 2025, 92% in 2024).

Areas with declines

- Last Viral Load Result dropped from 87% to 77%.
- Last Viral Load Date decreased from 81% to 69%.
- Next Appointment fell from 81% to 68%.
- Last ARV Date declined from 78% to 74%.
- Last Clinic Date slightly reduced from 80% to 77%.

Persistently weak indicators

- First ARV Date stayed unchanged at 64%.
- First Clinic Date improved to 63% but remains suboptimal.

The First CD4 date and result were first audited during the 2025 Corrective Data Quality Audit; therefore, these indicators were not included in the overall score. However, they received suboptimal scores of 60% and 65%, respectively, indicating the need for improvement.

Key Areas for Action

- Addressing historical indicator inaccuracies.
- Tracking records that need to be updated on a clinic day to reduce missed entries.
- Formalizing transfers on TSIS 2.0 for clients accessing care at other sites.
- Addressing all viral load and CD4 date and result inconsistencies.
- Improvement in the data protection of patient electronic records.

ACTION PLANS

Strategic Information Component HIV/STI/TB Unit Corrective Action Plan

Table 1: National-level Corrective Actions

HSTU	Findings	Action Required	Time Period	Remediation/ Actions to be taken	Status	Responsible Personnel
Strategic Information Component	Challenges with recording viral load, CD4 date and results on TSIS.	Conduct sensitization sessions outlining data entry practices for viral load and CD4 date and results.	February 2026	Sensitization session included in Supportive Supervision and Data Management meetings with Treatment Care and Support Officers.	Supportive Supervisions have been scheduled to discuss audit findings and develop a way forward.	Strategic Information Component
Strategic Information Component	Concerns surrounding the data protection of patients' electronic records.	Sensitization on the Data Protection Act and practices.	January- March 2026	Sensitization session included in Supportive Supervision and Data Management meetings with Treatment Care and Support Officers.	Supportive Supervisions have been scheduled to discuss audit findings and develop a way forward.	Strategic Information Component

Treatment Sites Data Quality Action Plan

Table 2: Site-level actions to improve data quality

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
Bellevue Hospital	Demographic information needs to be revisited. 25% of records had inaccuracies in the date of birth.	• Routine data review of patient files to correct date of birth inaccuracies. • Liaise medical records to highlight errors in demographic information for corrections to be made on the patient's record.	To begin February 2026	• Data Review activities with site staff, the Strategic Information Component focal point, and Medical Records to highlight errors in demographic information and make corrections on the patients' records. • The Medical Records Unit will pull volume one of each patient's file to see if the date of birth was recorded on admission. • Dr Williams will write to the	• Site staff • Strategic Information Component

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
				Medical Records Unit to request all twenty (20) patients' medical records for review and corrective action.	
	The First and Last Clinic Date for each patient must correlate with the Clinician's first physical exam at the initiation of HIV care across sites. 88% and 38%, respectively, had inaccuracies.	Routine data review of patient files to correct first and last clinic date inaccuracies.	To begin February 2026	Liaise with the Medical Records Unit to review the previous volume of patients' records to correct inaccuracies.	Site staff
	The First and Last ARV Date for each patient must be revisited. 88% and 38%, respectively, had inaccuracies.	Routine data review of patient files to rectify First and Last ARV Date inaccuracies.	To begin February 2026	• Liaise with the Medical Records Unit to review the previous volume of patients' records and rectify the first ARV date inaccuracies. • Last ARV inaccuracies will	Site staff

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
				be corrected on TSIS after review of the most recent intake tool.	
	Inaccurate First CD4 date and results entered on TSIS. 88% and 75%, respectively, had inaccuracies.	Routine data review of patient files to correct the first CD4 inaccuracies.	To begin February 2026	Liaise with the Medical Records Unit to review the previous volume of patients' records to correct the first CD4 date and results entered on TSIS.	Site Staff
	Patients who are no longer being treated by the site are still on their TSIS register.	Routine data review of patient data on TSIS to conduct data reconciliation exercises.	Completed November 2025	This has largely been corrected. However, the site will liaise with the strategic information component to conduct data reconciliation as needed. This has been actioned.	Site Staff
	Inaccurate Last Viral Load date and result entered in TSIS. 38% and 25%, respectively, had inaccuracies.	Routine data review of patient files to correct the Last Viral Load inaccuracies.	To begin February 2026	Routine data review of patient files to correct the Last Viral Load inaccuracies.	Site Staff

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	Electronic documents patient information are not consistently password-protected.	Ensure patient data in electronic format is consistently password-protected.	Ongoing	Ensure patient data in electronic format is consistently password-protected.	Site staff
	The Next Appointment Date for each patient must be revisited. 38% had inaccuracies.	Routine data review of patient files to rectify Next Appointment Date inaccuracies.	Ongoing	Data review activities will be conducted monthly to ensure that the correct appointment date is transcribed from the intake tool to TSIS in a timely manner.	Site Staff
St Ann's Bay Health Centre	Demographic information needs to be revisited. Six per cent of records had inaccuracies in the date of birth.	- Routine data review of patient files to correct date of birth inaccuracies. - Liaise with the Medical Records Unit to highlight errors in demographic information for corrections to be made on the patient's record.	Completed on December 1, 2025.	Liaise medical records to highlight errors in demographic information for corrections to be made on the patient's record.	Treatment Care and Support Officer(s) and team lead. Assisted by the Medical Records Unit. Above to be assisted by medical record personnel.
	The First Clinic Date for each patient must correlate with the Clinician's first physical exam at the initiation of HIV care across sites,	Routine data review of patient files to correct first clinic date inaccuracies.	Ongoing	Data Remediation activities will be conducted monthly to revise medical records specifically for clients initiated on ART on the ward.	Data Entrant guided by Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	27% of records had inaccuracies.				
	The First ARV Date for each patient must be revisited, 20% of records had inaccuracies.	Routine data review of patient files to rectify First ARV Date inaccuracies.	Ongoing	Monthly data quality review of patient medical records to address First ARV Date discrepancies.	Treatment Care and Support Officer(s)
	Inaccurate First CD4 date and results entered on TSIS, 28% and 25%, respectively.	Routine data review of patient files to correct the first CD4 inaccuracies.	Ongoing	Monthly data quality review of patient medical records to address First CD4 date and results discrepancies.	Data Entrant and Treatment Care and Support Officer(s)
	Patients who are no longer being treated by the site are still on their TSIS register.	Routine data review of patient data on TSIS to conduct data reconciliation exercises.	Ongoing	To revisit TSIS monthly and submit patients to be removed to the Strategic Information Component of the HIV /STI/TB Unit.	Data Entrant and Treatment Care and Support Officer(s)
	Inaccurate Last Viral Load date entered in TSIS, 35% of records had inaccuracies.	Routine data review of patient files to correct the Last Viral Load inaccuracies.	Ongoing	Monthly data quality review of patient medical records to address Last Viral Load date discrepancies.	Data Entrant and Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	Psychosocial updates are partially being entered in TSIS.	Increase psychosocial updates in TSIS.	Ongoing	Psychosocial team members to ensure information is accurately captured in TSIS.	Data Entrant and Treatment Care and Support Officer(s) And the Psychosocial team
	Electronic documents with patient information are not consistently password-protected.	Ensure patient data in electronic format is consistently password-protected.	Ongoing	Sensitization of all team members sharing sensitive information, and to advocate for properly locked filing cabinets and shredders for non-electronic documents.	Site team that shares sensitive information
St Ann's Bay Hospital- Paediatric	The First and Last ARV Date for each patient must be revisited, 35% of records had inaccuracies.	Routine data review of patient files to rectify First ARV Date inaccuracies.	Ongoing	To notate correct information for patients transferred to the site, for first and last visit data to be copied and placed in all volumes of records.	Health Records and PMTCT team
	Inaccurate First CD4 date and results entered on TSIS, 29% and 35%, respectively.	Routine data review of patient files to correct the first CD4 inaccuracies.	Ongoing	To notate correct information for patients transferred from external sites and pull information from archived records/volume 2	Health Records and PMTCT team

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
				records to reconcile data	
	Patients who are no longer being treated by the site are still on their TSIS register.	Routine data review of patient data on TSIS to conduct data reconciliation exercises.	Ongoing	List to be resubmitted to the Strategic Information Component of the HIV /STI/TB Unit for removal/ transfer.	PMTCT team
	Inaccurate Last Viral Load date and result entered in TSIS, 24% and 35% records had inaccuracies, respectively.	Routine data review of patient files to correct the Last Viral Load inaccuracies.	Ongoing	Monitor data entry of lab results in TSIS to ensure that the correct date is entered.	PMTCT team and Treatment Care and Support Officer(s)
	Inaccurate next appointment date entered in TSIS, 29% records had inaccuracies.	- Establish clear communication between medical records staff and doctors to verify the accuracy of the next appointment date before entering data into TSIS. - Routine data review of patient files to correct the next appointment date inaccuracies.	Ongoing	Re-sensitization of clinicians and medical records staff to correct data entry practices.	Clinicians, PMTCT nurse, Treatment Care and Support Officer(s)
	Psychosocial updates are partially being entered in TSIS.	Increase psychosocial updates in TSIS.	Ongoing	Psychosocial team members to ensure information is accurately captured in TSIS.	Social worker, case manager

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	Electronic documents patient information are not consistently password-protected.	Ensure patient data in electronic format is consistently password-protected.	Ongoing	Sensitization of all team members sharing sensitive information and advocating for properly locked filing cabinets and shredders for non-electronic documents.	Site team that shares sensitive information
May Pen Hospital Paediatrics	Demographic information needs to be revisited. 11% of records have inaccuracies in the date of birth.	• Routine data review of patient files to correct date of birth inaccuracies. • Liaise medical records to highlight errors in demographic information for corrections to be made on the patient's record.	Completed	Liaise with the Medical Records Unit to highlight errors in demographic information for corrections to be made on the patient's record.	Treatment Care and Support Officer(s)
	The First Clinic Date for each patient must correlate the Clinician's first physical exam at the initiation of HIV care across sites.	Routine data review of patient files to correct first clinic date inaccuracies	Ongoing	Monthly data quality review of patients' medical records to address First Clinic Date discrepancies.	Treatment Care and Support Officer(s)/ Data team
	The First ARV Date for each patient must be revisited—33%	Routine data review of patient files to rectify First ARV Date inaccuracies	Ongoing	Monthly data quality review of patient medical records to	PMTCT Nurse

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	of records have inaccuracies.			address First ARV Date discrepancies.	
	Inaccurate First CD4 date and results entered on TSIS.	Routine data review of patient files to correct the first CD4 inaccuracies.	Ongoing	Monthly data quality review of patient medical records to address First CD4 date and results discrepancies.	Treatment Care and Support Officer(s)/ Data team & PMTCT Nurse
	Patients who are no longer being treated by the site are still on their TSIS register.	Routine data review of patient data on TSIS to conduct data reconciliation exercises.	Ongoing	Review TSIS monthly for patients to be removed to the Strategic Information Component of the HIV /STI/TB Unit.	Treatment Care and Support Officer(s)/Data team & PMTCT Nurse
	Psychosocial updates are not being updated routinely in TSIS.	Ensure psychosocial updates are entered routinely in TSIS.	Ongoing	Psychosocial team members to ensure information is accurately captured in TSIS.	Case manager

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
Jamaica AIDS Support for Life - Kingston	The First ARV Date for each patient must be revisited, 26% of records had inaccuracies.	Routine data review of patient files to correct the first ARV date inaccuracies.	• Ongoing • To be done Quarterly	• Work with the Strategic Information Component to ensure that merged patient data files reflect correct information. • Conduct quarterly data quality checks by reviewing at least 10% of patient medical records to verify the accuracy of First ARV Date entries.	• M&E manager • Site staff
	Inaccurate First CD4 date and results entered on TSIS, 42% and 40%, respectively.	Inaccurate First CD4 date and results entered on TSIS, 42% and 40%, respectively.	Ongoing	Ensure that CD4 results from onsite testing (PIMA) are recorded in the medical records and on TSIS	Site staff
Jamaica AIDS Support for Life -Montego Bay	Inaccurate First CD4 date entered on TSIS, 27%	Inaccurate First CD4 date entered on TSIS, 27%.	Ongoing	Ensure that CD4 results from onsite testing (PIMA) are recorded in the	Site staff

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
				medical records and on TSIS.	
	Inaccurate Last Viral Load date and result entered in TSIS, 36% and 23%, respectively.	Inaccurate Last Viral Load date and result entered in TSIS, 36% and 23%, respectively.	Ongoing	Review records monthly to ensure that physical copies of viral load results are placed in all patient files	Site staff
Comprehensive Health Centre - Paediatric	The First Clinic Date for each patient must correlate the Clinician's first physical exam at the initiation of HIV care across sites, 24% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to identify and correct inaccuracies for the First Clinic Date.	- Ongoing - January – February 2026	- Conduct DQA for all clients retained in care quarterly. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional)
	The Last Clinic Date for each patient must correlate the Clinician's last physical exam for HIV care across sites, 24% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to identify and correct inaccuracies for the Last Clinic Date.	- Ongoing - January – February 2026	- Conduct DQA for all clients retained in care quarterly. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	The Last ARV Date for each patient must be revisited, 24% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to identify and correct inaccuracies in the Last ARV Date.	- Ongoing - January – February 2026	- Conduct DQA for all clients retained in care quarterly. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional)
	The Next Appointment Date for each patient must be revisited, 24% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to verify the accuracy of Next Appointment Dates.	- Ongoing - January – February 2026	- Conduct DQA for all clients retained in care quarterly. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional)
	Inaccurate Last Viral Load date and result entered in TSIS, 29% and 24%, respectively.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the Last Viral Load inaccuracies.	- Ongoing - January – February 2026	- Conduct DQA for all clients retained in care quarterly. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	Inaccurate First CD4 date and results entered on TSIS, 35%, each.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the First CD4 date and results inaccuracies.	Ongoing	- Review DISA monthly for CD4 results. - Update TSIS 2 CD4 results	Treatment Care and Support Officer(s) /Data Entry Officer
Spanish Town Hospital - Paediatric	The First Clinic Date for each patient must correlate the Clinician's first physical exam at the initiation of HIV care across sites, 38% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to identify and correct inaccuracies in the First Clinic Date.	- January – February 2026 - Ongoing	- Conduct an internal audit of all medical records to ensure corrections and updates. - Review of appointments on System One and appointment book monthly to ensure that records for all clients seen are updated on TSIS 2	Treatment Care and Support Officer(s)
	The First ARV Date for each patient must be revisited, 38% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify First ARV Date inaccuracies.	- January – February 2026 - Ongoing	- Conduct an internal audit of all medical records to ensure corrections and updates. - Review of appointments on	Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
				System One and appointment book monthly to ensure that records for all clients seen are updated on TSIS 2.	
	The Last Clinic Date for each patient must correlate with the Clinician's last physical exam for HIV care across sites, 62% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct Last Clinic Date inaccuracies.	- January – February 2026 - Ongoing	- Conduct an internal audit of all medical records to ensure corrections and updates. - Review of appointments on System One and appointment book monthly to ensure that records for all clients seen are updated on TSIS 2.	Treatment Care and Support Officer(s)
	The Last ARV Date for each patient must be revisited, 62% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify the Last ARV date.	- January – February 2026 - Ongoing	- Conduct an internal audit of all medical records to ensure corrections and updates. - Review of appointments on	Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
				System One and appointment book monthly to ensure that records for all clients seen are updated on TSIS 2.	
	The Next Appointment Date for each patient must be revisited, 69% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify Next Appointment Date inaccuracies.	- January – February 2026 - Ongoing	- Conduct an internal audit of all medical records to ensure corrections and updates. - Review of appointments on System One and appointment book monthly to ensure that records for all clients seen are updated on TSIS 2	Treatment Care and Support Officer(s)
	Inaccurate First CD4 date and results entered on TSIS, 69%, each.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the first CD4 inaccuracies.	- January – February 2026 - Ongoing	- Conduct an internal audit of all medical records to ensure corrections and updates. - Review of appointments on	Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
				System One and appointment book monthly to ensure that records for all clients seen are updated on TSIS 2	
	Inaccurate Last Viral Load date and result entered in TSIS, 23%, each.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the Last Viral Load inaccuracies.	- January – February 2026 - Ongoing	- Conduct an internal audit of all medical records to ensure corrections and updates. - Review of appointments on System One and appointment book monthly to ensure that records for all clients seen are updated on TSIS 2	Treatment Care and Support Officer(s)
Linstead Health Centre	Inaccurate First CD4 date entered on TSIS, of 27%.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the First CD4 inaccuracies.	January – March 2026	10% of records (medical records & TSIS) to be audited. - Review medical records and make the necessary corrections.	Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	Inaccurate Last Viral Load date entered in TSIS, of 32%.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the Last Viral Load inaccuracies.	January – March 2026	10% of records (medical records & TSIS) to be audited. - Review medical records and make the necessary corrections.	Treatment Care and Support Officer(s)
National Chest Hospital	The First Clinic Date for each patient must correlate with the Clinician's first physical exam at the initiation of HIV care across sites, 36% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct First Clinic Date inaccuracies.	Ongoing	10% of records (medical records & TSIS) to be audited quarterly. - Update TSIS 2 with CD4 results.	Treatment Care and Support Officer(s) Adherence Counsellor
	The First ARV Date for each patient must be revisited, 33% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify First ARV Date inaccuracies.	Ongoing	10% of records (medical records & TSIS) to be audited quarterly. - Update TSIS 2 CD4 results.	Treatment Care and Support Officer(s) Adherence Counsellor
	Inaccurate First CD4 date and results entered on TSIS, 36% and 33%, respectively.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records	Ongoing	10% of records (medical records & TSIS) to be audited quarterly.	Treatment Care and Support Officer(s) Adherence Counsellor

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
		to correct the First CD4 inaccuracies.		Update TSIS 2 CD4 results.	
	Inaccurate Last Viral Load date entered in TSIS, of 29%.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the Last Viral Load inaccuracies.	Ongoing	10% of records (medical records & TSIS) to be audited quarterly. - Update TSIS 2 with CD4 results.	Treatment Care and Support Officer(s) Adherence Counsellor
Comprehensive Health Centre - Adult	The First Clinic Date for each patient must correlate with the Clinician's first physical exam at the initiation of HIV care across sites, 27% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct First Clinic Date inaccuracies.	Ongoing	40 medical records to be audited monthly	Treatment Care and Support Officer(s)
	The First ARV Date for each patient must be revisited, 23% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify First ARV Date inaccuracies.	Ongoing	40 medical records to be audited monthly	Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	Inaccurate First CD4 date and results entered on TSIS, 32% and 24%, respectively.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the First CD4 inaccuracies.	Ongoing	40 medical records to be audited monthly	Treatment Care and Support Officer(s)
	Inaccurate Last Viral Load date entered in TSIS, of 21%	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the Last Viral Load inaccuracies.	Ongoing	40 medical records to be audited monthly	Treatment Care and Support Officer(s)
Windward Road Health Centre	The First ARV Date for each patient must be revisited, 21% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify First ARV Date inaccuracies.	Ongoing	10% medical records to be audited.	Treatment Care and Support Officer(s)
	Inaccurate First CD4 date and results entered on TSIS, 26% and 24%, respectively.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the First CD4 inaccuracies.	Ongoing	10% medical records to be audited.	Treatment Care and Support Officer(s)

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
Maxfield Park Health Centre	Inaccurate Last Viral Load date entered in TSIS, of 26%.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the Last Viral Load inaccuracies.	Ongoing	10% medical records to be audited	Treatment Care and Support Officer(s)/AC
	Inaccurate First CD4 date and results entered on TSIS, of 40% and 34%, respectively.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the First CD4 inaccuracies.	Ongoing	10% medical records to be audited	Treatment Care and Support Officer(s)/AC
Bustamante Hospital for Children	The Last Clinic Date for each patient must correlate the Clinician's last physical exam for HIV care across sites, 38% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct Last Clinic Date inaccuracies.	To begin January - February 2026	- Conduct DQA for all clients retained in care. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional) Officer(s)/Data Entry Officer
	The Last ARV Date for each patient must be revisited, 38% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify Last ARV Date inaccuracies.	To begin January - February 2026	- Conduct DQA for all clients retained in care. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional) Officer(s)/Data Entry Officer

Treatment Site/ Parish/ Institution	Findings	Action Required	Status	Remediation/ Actions to be taken	Responsible Personnel
	The Next Appointment Date for each patient must be revisited, 44% records had inaccuracies.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to rectify Next Appointment Date inaccuracies.	To begin January - February 2026	- Conduct DQA for all clients retained in care. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional Officer(s)/Data Entry Officer
	Inaccurate First CD4 date entered on TSIS, of 25%.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the First CD4 inaccuracies.	To begin January - February 2026	- Conduct DQA for all clients retained in care. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional Officer(s)/Data Entry Officer
	Inaccurate Last Viral Load date and result entered in TSIS, 44% and 31%, respectively.	- Review records to correct data quality audit findings. - Conduct monthly data quality checks by reviewing at least 10% of patient medical records to correct the Last Viral Load inaccuracies.	To begin January - February 2026	- Conduct DQA for all clients retained in care. - DQA of medical records for all clients seen each month	Treatment Care and Support Officer(s) (Parish Regional Officer(s)/Data Entry Officer

Facility Information

Table 3: DQA Audit Tool

Name of Institution						
Parish						
Region:						
Date of Evaluation						
Information on the participants who conducted / participated in the monitoring visit	Name of Reviewer				Position	
Date on which the sample was generated		The date the random sample was taken			Number of patients ON ART at the end of the last reporting period receiving ART	
The number of patients required to be reviewed						

The results are presented in two ways:

- Firstly, looking at the percentage of **matching indicators** nationally, regionally or by site.
- Secondly, looking at the **evaluation of items**, which is an *average score based on 5 areas* evaluated or observed throughout the audit, four of which are derived from the organizational evaluation (questionnaire).

A. Matching Indicators

The tool used throughout the audit indicates when correct matches are made by highlighting matching items in green. The collation of matching indicators can be seen in the example below, where the ten items crosschecked between the TSIS database (pre-populated fields) and health records (data to be entered from health records) are summarized.

PRE-POPULATED FIELDS											DATA TO BE ENTERED FROM DOCKET									
Record #	Registration Date	Docket Number	Sex	Birth Date	Next Appt.	Last Clinic Date	First ARV Date	Last ARV Date	Last Viral Load Date	Last Viral Load Result	Registration Date	Docket Number	Sex	Birth Date	Next Appt.	Last Clinic Date	First ARV Date	Last ARV Date	Last Viral Load Date	Last Viral Load Result
240	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data
254	data	data	data	data	data	data	data	data	data	data	data	data	data	data	no match	data	data	data	data	data
326	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data
699	data	data	data	data	data	data	data	data	data	data	no match	data	data	data	data	data	data	data	data	data
724	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data
745	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data
777	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data
1281	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	no match	data	no match	no match
1292	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data
1301	data	data	data	data	data	data	data	data	data	data	data	data	data	data	no match	data	data	data	data	data

Therefore, the results would be as follows;

Indicators	% Match
• Registration date	90%
• Health record number	100%
• Sex	100%
• Date of birth	100%
• Next appointment	80%
• Last clinic date	100%
• First ARV date	90%
• Last ARV date	100%
• Last viral load date	90%
• Last viral load result	90%

B. Evaluation of Items

- Proportion of health records that have been fully matched with TSIS.** Further to indicating when correct matches are made by highlighting matching items in green, the DQA tool also indicates “TRUE” under the column ‘All Elements Fully Matched’ when the entire row has been filled with correct matches. Where the field has both correct and mismatched items, the tool will indicate “FALSE”. This item collates all the health records for a particular site that have been matched as TRUE. For example, if only 6 health records of an overall 10 health records reviewed are TRUE, the score will be 60% (6/10). See below.

PRE-POPULATED FIELDS											DATA TO BE ENTERED FROM DOCKET										All Elements Fully Matched	Number of Matches
Record #	Registration Date	Docket Number	Sex	Birth Date	Next Appt.	Last Clinic Date	First ARV Date	Last ARV Date	Last Viral Load Date	Last Viral Load Result	Registration Date	Docket Number	Sex	Birth Date	Next Appt.	Last Clinic Date	First ARV Date	Last ARV Date	Last Viral Load Date	Last Viral Load Result		
240	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	TRUE	10
254	data	data	data	data	data	data	data	data	data	data	data	data	data	data	no match	data	data	data	data	data	FALSE	9
326	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	TRUE	10
699	data	data	data	data	data	data	data	data	data	data	no match	data	data	data	data	data	data	data	data	data	FALSE	9
724	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	TRUE	10
745	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	TRUE	10
777	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	TRUE	10
1281	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	no match	data	no match	no match	FALSE	7
1292	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	data	TRUE	10
1301	data	data	data	data	data	data	data	data	data	data	data	data	data	data	no match	data	data	data	data	data	FALSE	9
																					6	94

- Accuracy of data entered in the tool.** This considers the overall number of items that match after review of all available health records at the site. For example, if the number of health records required to be reviewed at a site was 10, each health record would require a review of 10 indicators, giving a total of 100 items to be reviewed. In the example below, 94 of a possible 100 matches are seen, giving a score of 94% (94/100).

DATA TO BE ENTERED FROM DOCKET										All Elements Fully Matched	Number of Matches
Registration Date	Docket Number	Sex	Birth Date	Next Appt.	Last Clinic Date	First ARV Date	Last ARV Date	Last Viral Load Date	Last Viral Load Result		
data	data	data	data	data	data	data	data	data	data	TRUE	10
data	data	data	data	no match	data	data	data	data	data	FALSE	9
data	data	data	data	data	data	data	data	data	data	TRUE	10
no match	data	data	data	data	data	data	data	data	data	FALSE	9
data	data	data	data	data	data	data	data	data	data	TRUE	10
data	data	data	data	data	data	data	data	data	data	TRUE	10
data	data	data	data	data	data	data	data	data	data	TRUE	10
data	data	data	data	data	data	no match	data	no match	no match	FALSE	7
data	data	data	data	data	data	data	data	data	data	TRUE	10
data	data	data	data	no match	data	data	data	data	data	FALSE	9
										6	94

- Evaluation of the organizational component.** Based on the components of the DQA questionnaire that focus on Evaluation of the organizational component, questions 1.1 – 1.5; responses to questions are rated in 3 ways: Yes = 100%, Partial = 50% and No = 0%. In the table below, the examples shown give an overall score of 80% based on reporting procedures, reporting responsibilities, receiving TSIS training and patient reconciliation.

№	Questions and Tasks	Evaluation Results	Rating	Comments	How to Answer
			Answers to questions: Yes No Partial		
1.	Evaluation of the organizational component	80%		State job title of staff being interviewed	The score is automatically generated
1.1	Reporting Procedures (Is TSIS data used to generate reports and used to do programme planning and site improvement?)	100%	Yes		Yes: procedures are in place and staff are adhering to them. Partial: there are prescribed procedures, but the staff does not follow them, or the procedures do not contain a complete algorithm of actions. No: no procedures.
1.2	Responsible person (Is someone directly responsible for preparing reporting data and ensuring data quality?)	100%	Yes		Yes: There is someone assigned, with an order/hierarchy of next responsible person in line. Partial: The order is not defined, but there is a person in charge. No: not specified by order. Staff do not know who these responsibilities are assigned to.
1.3	Interchangeability of persons (In the absence of a responsible person to prepare reports and ensure data quality, is there someone assigned to complete the task?)	0%	No		Yes: There is someone assigned, with an order/hierarchy of next responsible person in line. Partial: The order is not defined, but there is a person in charge. No: not specified by order. Staff do not know who these responsibilities are assigned to.
1.4	Training on HIV / AIDS in Jamaica (Did you receive TSIS2.0 related HIV training?).	100%	Yes		Yes: has completed training (training, workplace, etc.), understands how to work with the system in accordance with their responsibilities. Partially: They have been trained but do not understand how to monitor and report. No: I was not trained.
1.5	Patient reconciliation within the institution, within the region (Are formal checks made on patients receiving services at the treatment site between institutions nationally to avoid duplication of persons in the report?)	100%	Yes		Yes: Patient reconciliation is done within the institution and within the region. Partially: Patient reconciliation is conducted within the facility but is not conducted within the region. No: no reconciliation.

- **The process of data collection.** Items 2.1 – 2.3.2 give a breakdown of how data collection is assessed using patient-level accounting, institution-level accounting and documentation.

No	Questions and Tasks	Evaluation Results	Rating	Comments	How to Answer
			ANSWER to questions: Yes No Partial		
2.	The process of data collection	89%			The score is automatically generated
2.1	Patient-level accounting	100%			The score is automatically generated
2.1.1	Updating patient information (Is demographic and identifying info eg. address, received during treatment visits being updated /recorded on the front pages of patients' primary records? If so, where exactly? From the point of view of rapid verification)	100%	Yes		Yes: fixed on the front page of the docket. Partial: Not recorded on the front page of the Cards, but displayed on separate pages. They are fixed, but not fully. No: Not fixed at all. N / A: Not verified.
2.1.3	Service (Is every service on an outpatient card recorded in full: treatment scheme, date of appointment, medication, etc? In terms of the sufficiency of information to enroll the service in the reporting forms)	100%	Yes		Yes: fixed in full. Partial: not fully fixed (for example: fixed source, date and name of the drug, not fixed number of drugs). No: Not fixed at all. N / A: Not verified.
2.2	Accounting at the institution level	67%			The score is automatically generated
2.2.1	Psycho-social updates (Database updates are made by each member of the psycho-social team and in the relevant sections eg. Psychologist, social worker, nutritionist etc?)	0%	No		Yes: all information is conducted and filled in a timely manner, without any gaps. In part: it is ongoing, but it is not filled regularly, there are gaps, not all the necessary information is contained. No: Not in progress. N / A: Not verified.
2.2.2	TSIS (Is updated in a timely manner and without missing data, in full: patient data, ART data, admission, prescription, etc.) Within 7 days.	100%	Yes		Yes: all information is conducted and filled in a timely manner, without any gaps. In part: it is ongoing, but it is not filled regularly, there are gaps, not all the necessary information is contained. No: Not in progress. N / A: Not verified.
2.2.3	Apart from the TSIS database does patient information exist in another electronic format eg. Excel file. If so how is it maintained/protected? (expound in comments)	100%	Yes		Yes: the lists are maintained and have sufficient information to prepare the reports. Partial: the lists are maintained but do not contain enough information to prepare reports. No: No lists are being maintained.
2.3	Updating data in accounting documents	100%			The score is automatically generated
2.3.1	Updating the service in the docket (Is a patient's status updated in a timely manner eg. Death, Transfers, Migrated etc.)	100%	Yes		Yes: fixed and updated on time and in full. Partial: Fixed and updated on time, but not in full. Or fixed and updated in full, but not on time. No: They are not fixed or updated at all. N / A: Not verified.
2.3.2	System in place for troubleshooting issues surrounding the utilization of TSIS at the treatment site.	100%	Yes		Yes: System is in place for troubleshooting issues Partial: No such system is assigned however, temporary fixes are made on case by case basis by knowledgeable staff. No: They are not fixed or updated at all. N / A: Not verified.

- **Preparation and storage of reports.** In accessing and handling sensitive patient information, in-particular where names may be shown, it is vital to always work with encrypted files. TSIS reports are predominantly shared in Excel format, therefore we seek to answer whether or not these Excel files are always encrypted with a secure password. Likewise, the computers used to access these files are also password protected.

№	Questions and Tasks	Evaluation Results	Rating	Comments	How to Answer
			Answers to questions: Yes No Partial		
3	Preparation and storage of reports	100%			<i>The score is automatically generated</i>
3.1	Are excel reports pulled from TSIS always secured with a password.	100%	Yes		Yes: On every instance reports are pulled or shared, passwords are assigned to protect patient data Partial : Passwords may be added only when sharing the file across devices. No: No protections are added. N / A: Not verified.
3.2	Are computers where excel files are stored always secured with a password	100%	Yes		Yes: Passwords are assigned to all computers utilizing the TSIS database Partial : Passwords may be added only when sharing the file across devices. No: No protections are added. N / A: Not verified.

